



MEFENAMIC ACID SUSPENSION

COMPOSITION:

Each 5ml contains
Mefenamic Acid 125mg

Methylparaben 0.1% w/v as preservative.

PRESENTATION:

White colour and strawberry flavor suspension.

INDICATIONS:

Relief of mild to moderate pain including headache, dental pain, postoperative pain and dysmenorrhea; in musculoskeletal and joint disorders such as osteoarthritis and rheumatoid arthritis and in children with fever and juvenile rheumatoid arthritis.

PHARMACOLOGY:

Mechanism of Action:

Mefenamic acid, an anthranilic acid derivative, is a nonsteroidal anti-inflammatory drug, although its anti-inflammatory properties have been stated to be minor. The effect of NSAIDs appear to be due to their inhibitory action on cyclooxygenases which are involved in the biosynthesis of prostaglandins. Prostaglandins have an important role in the production of pain, inflammation, and fever and NSAIDs therefore find their main use as analgesics, anti-inflammatory agents, and antipyretics.

Pharmacokinetics:

Mefenamic acid is absorbed from the gastrointestinal tract. Peak plasma concentrations occur about 2 to 4 hours after ingestion. The plasma elimination half-life reported to be 2 to 4 hours. Mefenamic acid is extensively bound to plasma proteins. Small amounts have been detected in breast milk. Over 50% of dose may be recovered in the urine, as unchanged drug or conjugates of mefenamic acid and its metabolites.

DOSAGE AND ADMINISTRATION:

Oral administration.

SHAKE WELL BEFORE USE.

For adult:

Initially 500mg (20ml) then 250mg (10ml), 6 hourly.

For children:

In children over 6 months: 25mg (1ml)/kg body weight daily in divided dose. Maximum for 7 consecutive days.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

CONTRAINDICATION:

In patients with inflammatory bowel disease, those suffering from peptic and / or intestinal ulceration, and those with renal or hepatic impairment.

PRECAUTION:

Mefenamic acid should be given with care to patients with asthma or bronchospasm, bleeding disorder, a history of peptic ulceration, a history of hypersensitivity to aspirin or other NSAID, hypertension, and impaired renal, hepatic or cardiac function. In patients suffering from dehydration and renal disease, particularly the elderly, concurrent therapy with other plasma protein-binding drugs may necessitate a modification in dosage. In menorrhagia, lack of response should alert the physician to investigate other causes. Some consider that blood counts should be performed during long-term therapy.

Cardiovascular Thrombotic Events:

Use of non-selective NSAIDs may be associated with an increased risk of cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or risk factors for cardiovascular disease, the lowest effective dose should be used for the shortest possible duration. There is no evidence that concurrent use of aspirin decreases the risk of cardiovascular thrombotic events associated with NSAIDs use.

Hypertension:

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAIDs treatment and at regular intervals thereafter.

Heart Failure:

Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events:

As with all NSAIDs, gastrointestinal discomfort and rarely serious gastrointestinal bleeding, ulceration or perforation can occur during treatment, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increase the risk of serious gastrointestinal adverse events.

Severe Skin Reactions:

NSAIDs may very rarely cause serious cutaneous adverse

events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use.

Serious skin reactions such as Generalised bullous fixed drug eruption (GBFDE) have been reported very rarely in association with the use of mefenamic acid. Patients should be advised of the signs and symptoms of serious skin reactions and Mefenamic acid should be discontinued at the first appearance of the skin rash, mucosal lesions or any other sign of hypersensitivity.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Use in Pregnancy and lactation:

Regular use of mefenamic acid during the 3rd trimester of pregnancy may result in closure of foetal ductus arteriosus in utero, and possibly in persistent pulmonary hypertension of the newborn. The onset of labor may be delayed and its duration increased. Therefore, the use of mefenamic acid in pregnant women is not recommended. Trace amounts of mefenamic acid may be present in breast milk and transmitted to the nursing infants. Therefore, mefenamic acid should not be taken by nursing mothers.

Newborn / Premature:

Not recommended for children below 6 months.

SIDE EFFECTS:

The most common side effects encountered with the use of mefenamic acid are gastrointestinal disturbances. Reversible haemolytic anaemia, diarrhoea and skin rashes may occasionally occur. If diarrhoea or a rash occurs, the medication should be withdrawn immediately. Patients on prolonged therapy should be kept under surveillance with particular attention to renal failure. Should this appear the therapy should be discontinued. Bronchospasm may be precipitated in patient suffering from, or with a previous history of bronchial asthma or allergic disease.

Skin and subcutaneous tissue disorders:

Generalised bullous fixed drug eruption (GBFDE)

DRUG INTERACTIONS:

Possible enhancement of the effects of oral anticoagulants and increased plasma concentrations of lithium, methotrexate, and cardiac glycosides. The antihypertensive effect of ACE inhibitors, betablocker, and diuretics may be reduced. Convulsions may occur due to an interaction with quinolones. Enhance the effects of phenytoin and sulphonylurea antidiabetics. The concomitant use of more than one NSAID should be avoided because of the increased of adverse effects. Risk of gastrointestinal bleeding and ulceration associated with mefenamic acid is increased when used with corticosteroids.

OVERDOSAGE & TREATMENT:

Mefenamic acid overdose has been associated with CNS toxicity, especially with convulsion. Coma has also been reported. Gastric lavage in the conscious patient and intensive supportive therapy where necessary. Activated charcoal has been shown to be powerful adsorbent for mefenamic acid and its metabolites.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN

JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in 60ml PET bottle.

Further information can be obtained from pharmacist, physician or the manufacturer.

MAL20013770AZ

Product Registration Holder & Manufactured By:



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